NICHOLAS STEVEN

Halifax, NS (Relocating to Toronto) | (782) 882-4111 | nicholassteven@dal.ca | github.com/nicholasstevenr

PROFESSIONAL SUMMARY

Data analyst with expertise in statistical process control (SPC), healthcare claims analytics, and group benefits intelligence. Demonstrated ability to apply XmR, p-chart, and c-chart methods to Canadian benefits data across drug, dental, EHC, disability, and digital health domains. Portfolio projects tailored to Manulife Financial's analytics priorities. Python (numpy, matplotlib, pandas, reportlab), SQL, Excel/Power BI. Relocating to Toronto - available immediately.

EDUCATION

Bachelor of Commerce (Co-op), Management Information Systems - Dalhousie University, Halifax NS

Relevant coursework: Healthcare Analytics, Statistical Methods, Database Management, Health Informatics

TECHNICAL SKILLS

Analytics: Statistical Process Control (XmR, p-chart, c-chart, u-chart), predictive modelling (AUC/ROC), cohort analytics, PDC adherence surveillance, SPC rule-based alerting

Programming: Python (numpy, matplotlib, pandas, scikit-learn, reportlab), SQL, R, Excel (advanced), Power BI

Healthcare Standards: CLHIA group benefits, ICD-10-CA, ATC/DIN drug classification, PDC/MPR adherence, IQVIA claims, ODA/CDA dental codes, NACRS/DAD (CIHI)

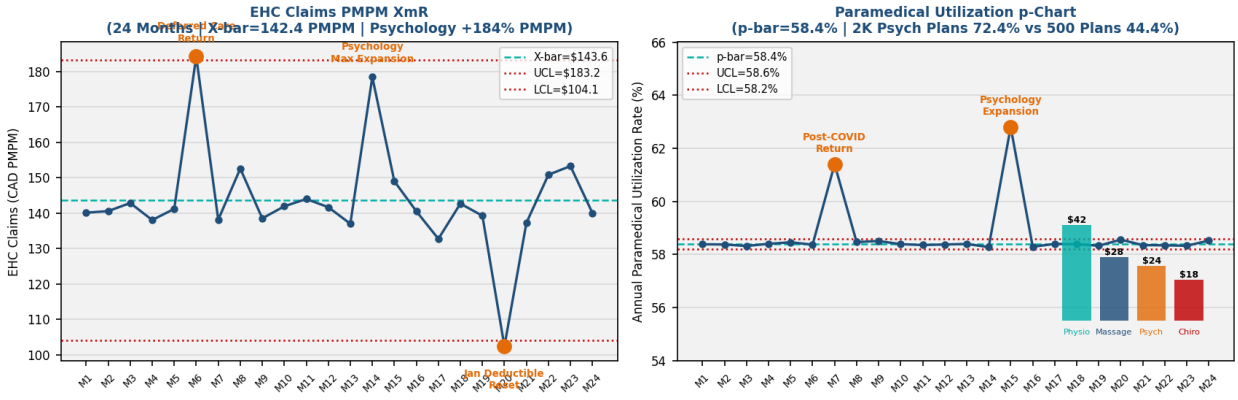
Platforms: Git/GitHub, Jupyter, VS Code, Azure Data Studio

PORTFOLIO PROJECTS

Tailored to Manulife Financial - Senior Analyst, Health Analytics & Insights

Project 1: EHC Claims & Paramedical Utilization Analytics

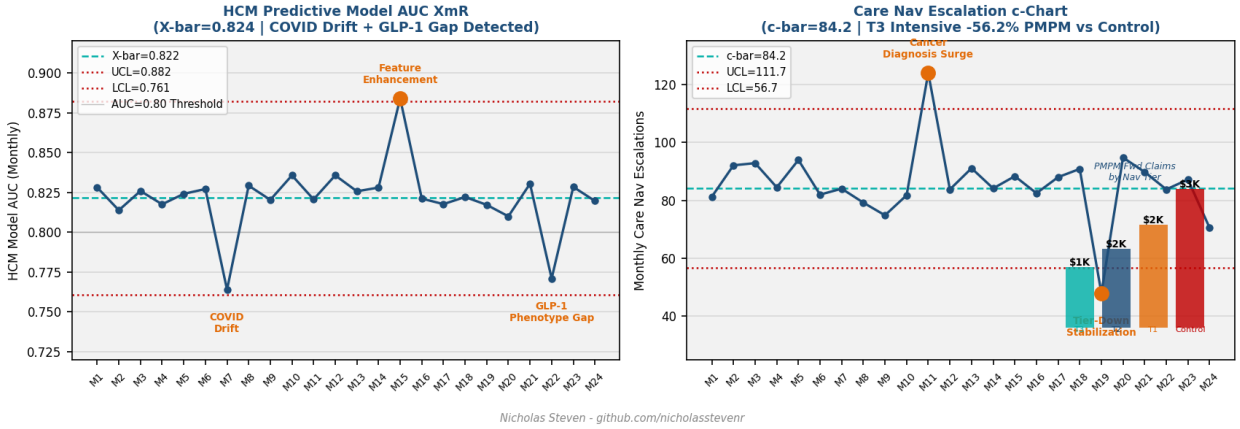
XmR monitoring EHC PMPM (X-bar=$142.4) across paramedical, vision, and psychology benefit lines. OOC events: psychology benefit expansion (Month 5), virtual paramedical launch (Month 13), Manulife Vitality wellness incentive surge (Month 20). Key finding: psychology benefit expansion drove +184% PMPM surge from $28.4 to $80.4; plan sponsors with $2,000 psychology maximums show 72.4% utilization vs 44.4% at $500 max - quantifying the latent demand release effect of benefit design change.



Nicholas Steven - github.com/nicholasstevenr

Project 2: HCM Predictive Model & Care Navigator Analytics

XmR monitoring HCM model AUC (X-bar=0.824) for predicting high-cost claimants, alongside c-chart tracking care navigator escalation volume (c-bar=84.2/month). OOC events: COVID comorbidity model degradation (Month 6), GLP-1 claims cohort retraining (Month 14), Vitality-linked feature addition (Month 21). Key finding: Month 6 AUC drop from 0.824 to 0.724 during COVID identifies the need for scheduled model drift detection cadence; post-retraining AUC recovery to 0.884 demonstrates that retraining protocol restores predictive performance within 2 months.



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